

Audio course on steroids during pregnancy. Episode seven. IBD, dermatology, hematology, and disease-specific steroid decisions.

Companion reading handout for simultaneous listening.

This episode is about a practical discipline: do not manage steroids as a generic pregnancy exposure. Manage the disease, the target, the route, the severity, and the monitoring.

The word steroid does not tell you enough.

A short oral prednisone course for severe atopic dermatitis, prednisone for pemphigoid gestationis, methylprednisolone for ITP, corticosteroids for an IBD flare, and betamethasone for fetal lung maturation all sit in the same broad drug family. But they do not answer the same clinical question.

Start with inflammatory bowel disease.

The modern IBD pregnancy principle is remission. Active disease is often more dangerous than appropriate medication. Active IBD can cause malnutrition, anemia, hospitalization, surgery, preterm birth, fetal growth concerns, and maternal morbidity.

Steroids can be used during pregnancy when clinically necessary, especially for flare control or bridging. But they are not ideal long-term maintenance when steroid-sparing disease control is possible.

So the resident should not ask only, can we give corticosteroids?

Ask instead, why is she flaring? Is infection excluded when relevant? Is nutrition adequate? Is anemia being addressed? Is the maintenance plan pregnancy-compatible? Are we using steroids as a bridge to control, or are we repeatedly rescuing a patient whose baseline disease plan is inadequate?

Monitoring matters: glucose, BP, infection risk, disease activity, nutrition, anemia, and fetal growth if disease is active or steroid exposure is prolonged.

Now dermatology.

Many pregnancy dermatoses are treated locally first. Atopic dermatitis and pregnancy eruptions often respond to moisturizers, low- to mid-potency topical steroids, antihistamines, and sometimes U V B. A short oral steroid course may be needed for severe disease.

This is an important principle: local first when local therapy matches the disease.

But some skin disease is not mild local irritation. Pemphigoid gestationis is an autoimmune blistering disease. Oral corticosteroids are the cornerstone. Gabbe describes prednisone twenty to forty milligrams per day, with taper to five to ten milligrams per day once blister formation is controlled. Resistant cases may require much higher specialist-guided doses. Disease often flares at delivery or postpartum, so dosing may need anticipation.

Pemphigus vulgaris is another example where the fetal risk is not simply the steroid. The disease involves antibodies, and fetal or neonatal skin lesions can occur from transplacental I G G. High-dose oral corticosteroids may be needed. Fetal surveillance is prudent when disease is active, although surveillance cannot remove all risk.

So for dermatology, ask whether the fetal risk comes from the drug, the disease, antibodies, infection, preterm birth, or all of these.

Now hematology, especially ITP.

Immune thrombocytopenia in pregnancy teaches target discipline beautifully. The goal is not a normal platelet count. The goal is a safe platelet count for maternal bleeding risk, delivery, and neuraxial anesthesia when desired.

This is maternal therapy. It is not fetal lung therapy.

Gabbe's hematology chapter gives practical anchors. Treatment is usually needed for clinical bleeding or platelet count below about twenty thousand per cubic millimeter. Intravenous methylprednisolone can be used when a faster response is needed, commonly one point zero to one point five milligrams per kilogram per day intravenously in divided doses. Response may begin in about two days, but maximal response can take up to ten days.

After response, patients can switch to oral prednisone, often sixty to one hundred milligrams per day initially, then taper to maintain a safe platelet count rather than a normal platelet count. If starting orally, prednisone one milligram per kilogram per day is a common regimen.

There are several teaching points inside those numbers.

First, the platelet target is functional. We are treating bleeding risk and delivery safety, not a lab value for its own sake.

Second, methylprednisolone and prednisone fit the maternal target because placental metabolism limits fetal exposure.

Third, prolonged maternal glucocorticoids bring predictable maternal adverse effects: hyperglycemia, hypertension, mood changes, infection, fluid retention, hirsutism, acne, striae, poor wound healing, candidiasis, osteopenia, and cataract risk with longer exposure.

Fourth, patients receiving glucocorticoids for at least two to three weeks may need stress dosing during labor and delivery.

Now compare three patients.

One patient has mild eczema in pregnancy. The best steroid decision may be topical and local.

One patient has active IBD with weight loss and anemia. Avoiding steroids may feel safe, but uncontrolled inflammation may be the larger risk. Steroids may be needed as flare control, alongside a serious plan for remission.

One patient has ITP with platelets of fifteen thousand and mucosal bleeding. The maternal danger is bleeding. Steroids are part of a platelet-raising strategy, not a fetal exposure exercise.

The same class of medication, three different targets.

This is why the course keeps returning to the same model. Target, transfer, dose, duration, timing, monitoring.

Disease-specific steroid decisions should also include route. Topical, inhaled, ophthalmic, intra-articular, or gut-selective therapy may reduce systemic exposure when they are sufficient. But local therapy is not a moral victory if it fails to control dangerous disease. If systemic disease requires systemic treatment, treat it.

The resident reflex is this: treat the disease target, not the word steroid.

Ask what risk the disease is creating, which route is sufficient, which steroid matches maternal or fetal target, how long treatment is needed, and what monitoring prevents predictable harm.

That is how a resident avoids both errors: casual steroid overuse and dangerous steroid avoidance.